Toddler Physical Aggression Usually Peaks and Declines

Physical aggression commonly rises through the second and third years and declines for most children; a smaller group remains elevated, and presence of hitting is not a violence forecast.

TO-HIGH-TRAJECTORY; MODERATE-INDIVIDUAL; LOW-VIOLENCE-FORECAST | STATUS: DRAFT | DOMAIN: BEHAVIOR, SOCIAL | REP

WHAT WE KNOW (Narrowest Supported Conclusion)

Physical hitting, grabbing, and related acts are common in toddlerhood. Tremblay et al. (2004) described trajectories of physical aggression in early childhood: a typical rise and later decline for most children, and a smaller group with persistently elevated frequency. Frequency, severity, context, and change over time are more informative than presence alone.

EVIDENCE AND CAUSAL ASSESSMENT

Longitudinal cohorts identify multiple trajectories. These designs support probabilistic pathways, not inevitability. Child characteristics, language, sibling and peer contexts, caregiver responses, and stress all participate. This is not a genetically informed attenuation package on this card. Parent-training efficacy belongs to Chapter 14 and is not this identification.

EFFECT SIZE AND PERSISTENCE

Most children reduce physical aggression after the toddler peak. Persistence is less common and more consequential. Aggression can change form rather than disappear.

BIAS, HETEROGENEITY, AND CONFOUNDING

GI attenuation: untested-in-GI (early physical aggression is moderately heritable with environmental moderation across typical cohorts; quantitative sibling/adoption attenuation for trajectory shapes remains untested).

Parent-report and observer measures differ. Sexed rating biases, socioeconomic stress, sleep, disability, and exposure to violence are intertwined with caregiver responses.

WHAT THIS DOES NOT MEAN (Strict Boundary Conditions)

Toddler hitting does not predict later violence as destiny. Ordinary aggression should not be ignored when it is severe, dangerous, persistent, or impairing. Aggression is not proof of poor parenting or a failed “no.”

WHAT ACTION IS REASONABLY JUSTIFIED

Developmentally informed monitoring and targeted support for persistent or dangerous patterns. Alarmist labels for common transient behavior do not follow.

VERIFIED SOURCES & DOI IDENTIFIERS

- Tremblay et al. (2004), Pediatrics